



OFFICIAL

Management of Thrombocytopenia in Pregnancy Guideline

1. Purpose

To Identify women with Thrombocytopenia in pregnancy, plan multidisciplinary management to improve maternal, fetal and newborn outcomes.

Thrombocytopenia is defined by a circulating platelet count $<150 \times 10^9/L$, affects 5-10% of all pregnancies. The platelet count rarely falls below 100, with approximately 1% of pregnant women having a platelet count below this threshold

2. Applicability

This guideline applies to the midwifery, nursing and medical disciplines within the Obstetrics and Gynaecology and Anaesthesia departments of Armada Health Service.

3. Guidelines

Physiological thrombocytopenia occurs in normal pregnancy, with an average decrease in platelet count of 10%, occurring mostly in the 3rd trimester. It is due to haemodilution or accelerated platelet destruction and normalises 24 -72 hours post-partum

Up to 10% of pregnancies are complicated with thrombocytopenia, characterised as:

- Mild $100-150 \times 10^9/L$
- Moderate $50-100 \times 10^9/L$
- Severe $< 50 \times 10^9/L$

Gestational and ITP are the most common causes of thrombocytopenia in pregnancy

3.1 Gestational thrombocytopenia

- Occurs in 5-9% of pregnancies
- Accounts for 70-80% of pregnancy-associated thrombocytopenia
- Asymptomatic (usually), occurs in third trimester (rarely late 2nd trimester)
- Platelet count $> 70 \times 10^9/L$, normalises post-partum
- No specific diagnostic test available, diagnosis of exclusion
- Is not associated with increased maternal haemorrhage nor fetal thrombocytopenia

3.2 Immune thrombocytopenia (ITP) (also known as Idiopathic thrombocytopenia purpura or autoimmune thrombocytopenia purpura)

- Rare, 3% of pregnancy-associated thrombocytopenia
- Associated with risk of maternal and fetal haemorrhage
- Thrombocytopenia occurs at any gestation, and may be $< 50 \times 10^9/L$

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- Is the most common cause of thrombocytopenia in first and second trimesters
- Requires multi-disciplinary management, including obstetrician, anaesthetist, haematologist / physician and neonatologist/paediatrician

3.3 Causes of Thrombocytopenia:

Pregnancy specific:

- Gestational thrombocytopenia
- Pre-eclampsia
- Haemolysis, Elevated Liver enzymes Low platelets (HELLP), Acute fatty liver of pregnancy

Non-pregnancy specific – increased destruction:

- ITP
- Thrombotic thrombocytopenia purpura
- Haemolytic uremic syndrome
- Disseminated intravascular coagulation
- Drug induced
- Viral infections e.g., HIV, HCV, EBV, CMV
- Hypersplenism

Non-pregnancy specific – Decreased production:

- Bone marrow disease
- Nutritional deficiency: B12, folate
- Liver disease
- Congenital thrombocytopenia

4. Management

4.1 Antenatal management:

In general, a platelet count >100 does not require intervention, though ongoing surveillance blood is warranted. The frequency of testing depends on the platelet count and gestational age:

- A full blood count (FBC) every four weeks is often adequate in early pregnancy.
- Fortnightly testing is generally performed from 32-34 weeks gestation
- Weekly counts advised when thrombocytopenia is more severe or if the clinical situation is unstable at any gestation age
- Send bloods for platelet function defects and platelet aggregation studies to evaluate acquired and inherited platelet function defects for moderate to severe thrombocytopenia.
- Continue routine GP Obstetrician appointments and perform investigations as above
- Refer to specialist Obstetrician.

4.2 Anaesthetic management:

- Neuraxial anaesthesia is generally considered low risk in the context of gestational thrombocytopenia with stable platelet counts greater than $70 \times 10^9 /L^3$.

- Neuraxial anaesthesia should be considered on a case-by-case basis when platelet counts are between $50-70^{10} \times 10^9/L$
- The risk of epidural hematoma for a platelet count of 0 to 49,000 is 11%, for 50,000 to 69,000 is 3%, and for 70,000 to 100,000 is 0.2%.”
- The incidence of spinal epidural haematoma is $< 1:150\ 000$ following epidural and $< 1: 200\ 000$ following spinal anaesthesia but increases in the setting of maternal thrombocytopenia.

A recent position statement paper suggests that a neuraxial anaesthesia be considered in women with a platelet count >70 with a normal coagulation profile and absence of other risk factors for bleeding².

4.3 Referral to anaesthetist:

Refer all women from outpatient clinics with platelet count <100 any time in pregnancy especially third trimester via an e-referral. Contact DA Obstetrics for a plan of management if woman with moderate to severe Thrombocytopenia presents to AAU.

4.4 Haematology Referral:

- Haematology referral can be made by either GP Obstetrician or Specialist Obstetricians, whoever sees the woman first.
- In the 3rd trimester, it is reasonable to refer a patient with a platelet count <100 .
- If >34 weeks, a phone call can be made to the Haematology registrar or consultant at FSH. Some of these cases might be able to be discussed via phone call, e.g., platelet count in 90's at 40 weeks in well woman.
- Before the third trimester, referrals with minor thrombocytopenia, 100-150 as long as the FBC report does not comment that there is platelet clumping
- Any woman with any concerning features and a low platelet count of any level at any stage
- Any woman with a platelet count that has been low outside of pregnancy
- Previous severe thrombocytopenia in pregnancy (Platelet count $<100 \times 10^9/L$)
- History of ITP

4.5 Labour management:

- Women with a platelet count <100 should generally deliver in a hospital with Obstetric and anaesthetic experience and a 24-access to laboratory testing and blood bank support.
- Significant bleeding is unlikely with either C-section or vaginal birth with a stable platelet count >50
- Decision between modes of delivery should be based on Obstetric factors. Mode of delivery in women with ITP is based on Obstetric indication
- Consider induction/ delivery in late gestation after 39 weeks for falling platelet count in consultation with anaesthetist
- Keep anaesthetist on call informed of the woman in labour with low platelets
- Site Intravenous cannula, send FBC, coagulation profile at start of labour

- Check platelets within four hours of neuraxial block being placed and before epidural catheter is removed.
- Minimise vacuum delivery, rotational forceps or difficult instrumental delivery where fetal thrombocytopenia is suspected due to risk of intracranial bleeding.
- Possible adverse effects of instrumental birth must be weighed against consequences of awaiting vaginal birth or alternatively of performing C-section with head deep in pelvis
- Aspirin is commonly prescribed in pregnancy and should not be withheld unless the platelet count is <50 or risk of bleeding is high

5. Post-natal advice

- Keep paediatrician informed of low platelets of mother.
- Avoid non-steroidal anti-inflammatories if platelet count <70
- Assess for the risk of venous thromboembolism (VTE). If at increased risk for VTE (e.g. caesarean section, antiphospholipid antibodies), consider appropriate thromboprophylaxis. Pharmacological thromboprophylaxis is generally safe if the platelet count is $>50 \times 10^9/L$ though treatment decisions must be individualised. Discuss with haematologist.
- Check FBC on day 1 postpartum. Further follow-up depends on clinical factors but a repeat count approximately 2 and 6 weeks postpartum are generally advised.
- Advise mother to follow up with her GP to check platelets in six weeks' time post-delivery.
- Refer to haematologist for further evaluation if persisting low platelet levels.

6. Roles and responsibilities

Role	Responsibility
Booking Midwife	Document booking platelet levels, necessary send referral if low booking platelets
GPO/ Specialist Obstetrician	Evaluate platelet levels and referral to anaesthetic and haematology
Anaesthetist	Provide plan for neuraxial block in labour

7. Compliance, monitoring and evaluation

Monitoring will be by audit of correct management of women with low platelet count in pregnancy.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.3
- Standard 5 – Action 5.11
- Standard 5 – Action 5.12
- Standard 5 – Action 5.13
- Standard 6 – Action 6.7
- Standard 8 – Action 8.4

9. References

1. Reese JA, Peck JD, Deschamps DR, et.al. Platelet Counts during Pregnancy. New England Journal of Medicine. 2018;379(1):32-43
2. Eslick R, Cutts B, Merriman E, et.al. HOW Collaboration position paper on the management of thrombocytopenia in pregnancy. ANZJOG. 2021;61(2):195-204
3. Melissa E. Bauer, DO, Katherine Arendt, MD<Yaakov Beilin et.al. The Society for Obstetrics Anaesthesia and Perinatology Interdisciplinary Consensus statement on Neuraxial procedures in Obstetric patients with Thrombocytopenia
4. Xu X, Zhang Y, Yu X, Huang Y. Pre-operative moderate thrombocytopenia is not associated with increased blood loss for low-risk caesarean section: a retrospective cohort study. BMC Pregnancy and Childbirth.2019;19 (1):269
5. Patient Blood Management Guidelines: Module 5: Obstetrics and Maternity:2015

10. Glossary

The following definitions are relevant to this policy.

Term	Definition
Thrombocytopenia	Platelet count <150x10 ⁹ /L
Neuraxial anaesthesia	Epidural or spinal anaesthesia

11. Document contact

Director Clinical Services

Enquiries relating to this guideline may be directed to:

Title: Head of Obstetrics & Gynaecology
 Department: Obstetrics and Gynaecology
 Email: sangeeta.mallabhat@health.wa.gov.au

12. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	23 02 2026	23 02 2029	Full review, minor updates only.

13. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dr Tania Strickland - Director Clinical Services
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